



Mahidol University
Faculty of Nursing

Health system and health policy

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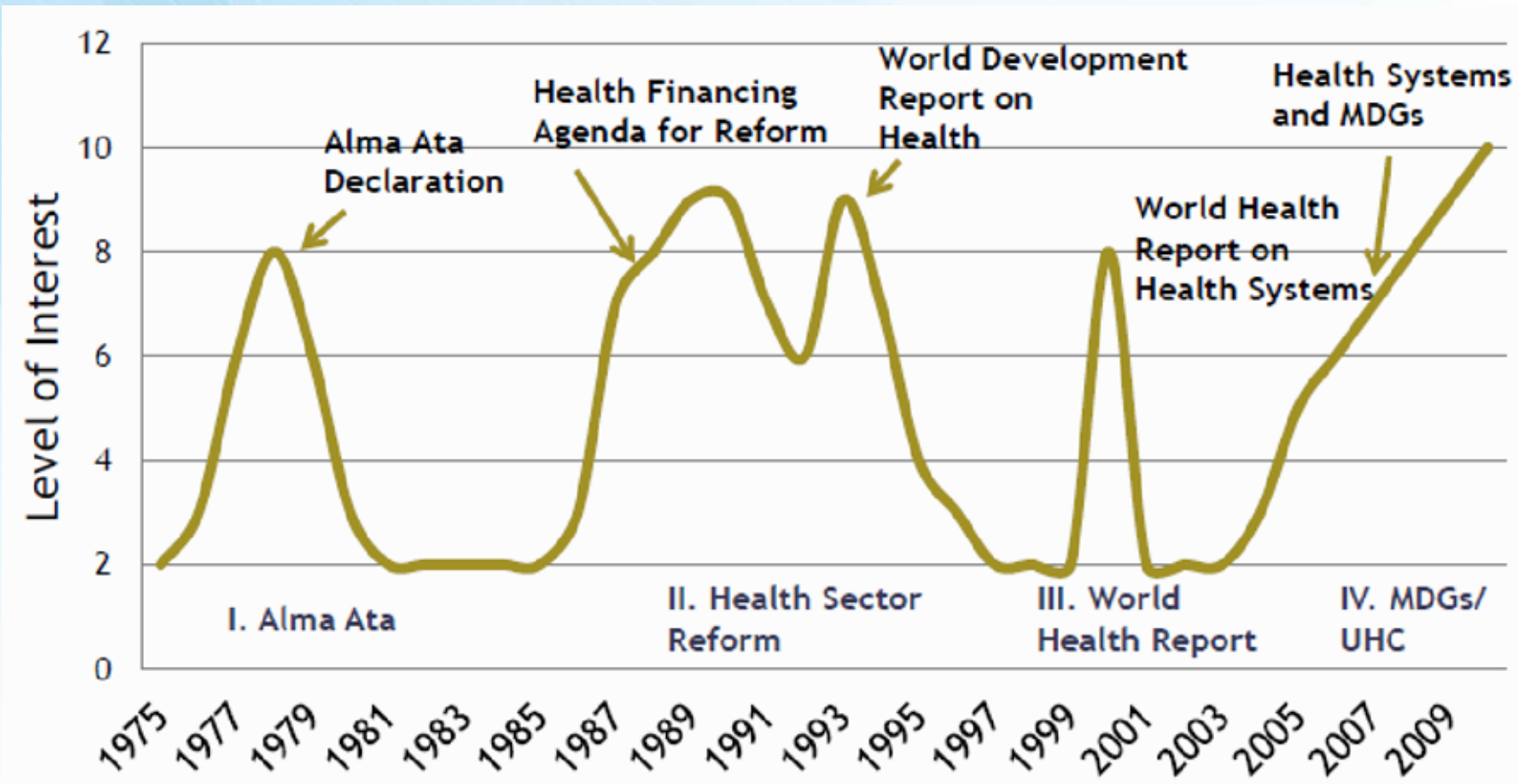
Outline

- Health system definition
- Evolution of Health Systems
- The six building blocks
- Access to Health Care
- Health policy definition
- Health policy analysis

Health system definition

- A health system consists of all organizations, people and actions whose primary intent is to promote, restore or maintain health
- Its goals are improving health and health equity in ways that are responsive, financially fair, and make the best, or most efficient, use of available resources

Evolution of Health Systems



Sara Bennett (2018), Health Systems: Frameworks, Theories, and Reform Models, Johns Hopkins University.

Evolution of Health Systems

Theme	Period	Lead proponent	Key elements
Alma Ata/primary health care	1978	WHO, governments	First-level services, community participation, equity and access, role of the state; multisectoral and holistic
Health sector reform	1987 forward	World Bank, USAID, UK Department for International Development	Increase efficiency, raise additional revenues, roll back the state, and make government more accountable; decentralization, insurance, hospital boards

Sara Bennett (2018), Health Systems: Frameworks, Theories, and Reform Models, Johns Hopkins University.

Evolution of Health Systems

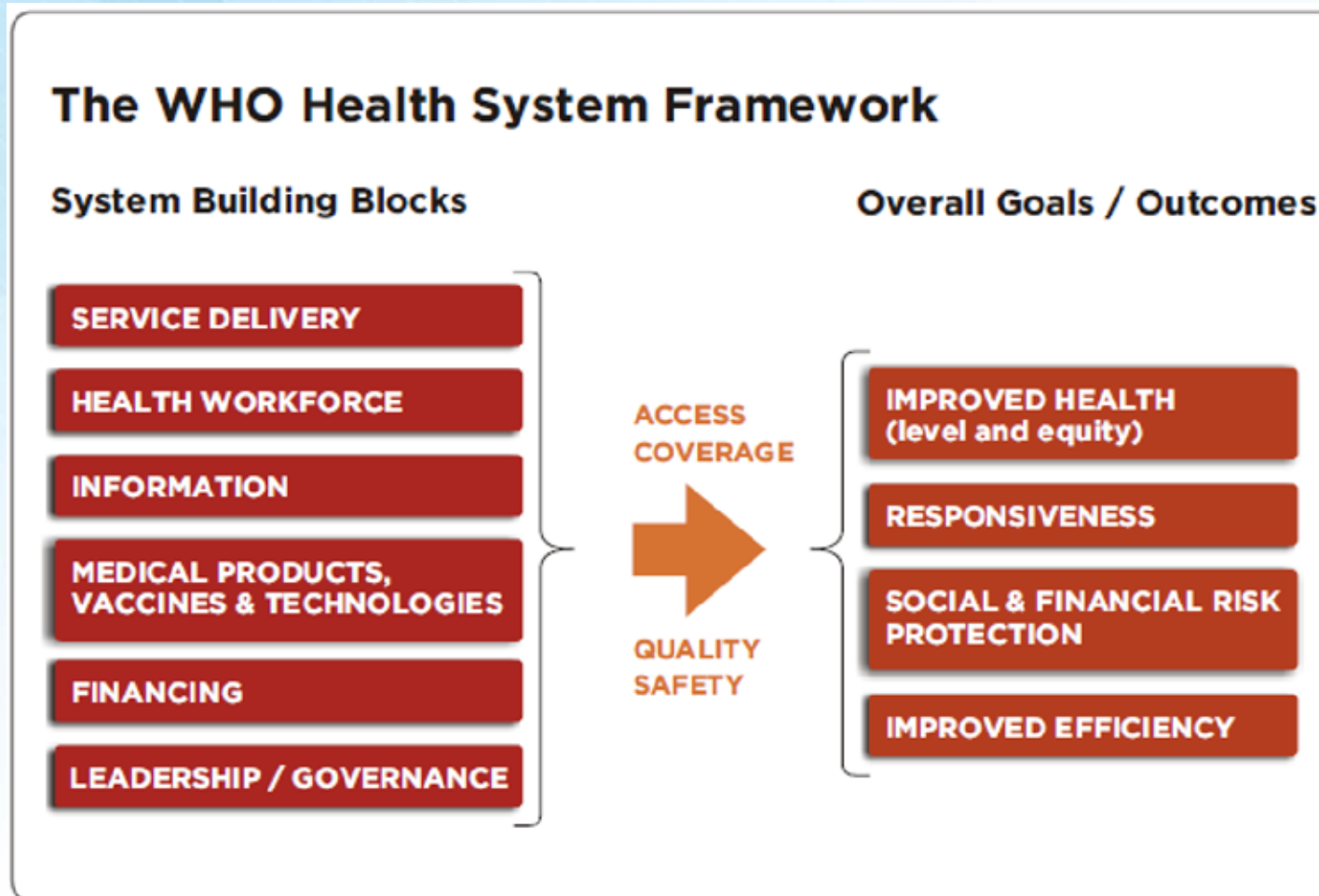
Theme	Period	Lead proponent	Key elements
Health system and MDGs	Mid-2000s forward	Global funds, WHO, bilaterals	Combine focus on high-priority diseases with measures to alleviate the health system bottlenecks; “diagonal approach”
Universal health coverage	Late 2000s and current	WHO, UN General Assembly	Providing financial protection and extending coverage

Health system building blocks

- The dynamic architecture and interconnectedness of the health system building blocks



The six building blocks



System building blocks

- Service delivery includes effective, safe, and quality personal and non-personal health interventions that are provided to those in need, when and where needed (including infrastructure), with a minimal waste of resources
- Health workforce: responsive, fair and efficient given available resources and circumstances, and available in sufficient numbers

System building blocks

- Health information: ensuring the production, analysis, dissemination and use of reliable and timely information on health determinants, health systems performance and health status
- Medical technologies: including medical products, vaccines and other technologies of assured quality, safety, efficacy and cost effectiveness, and their scientifically sound and cost-effective use

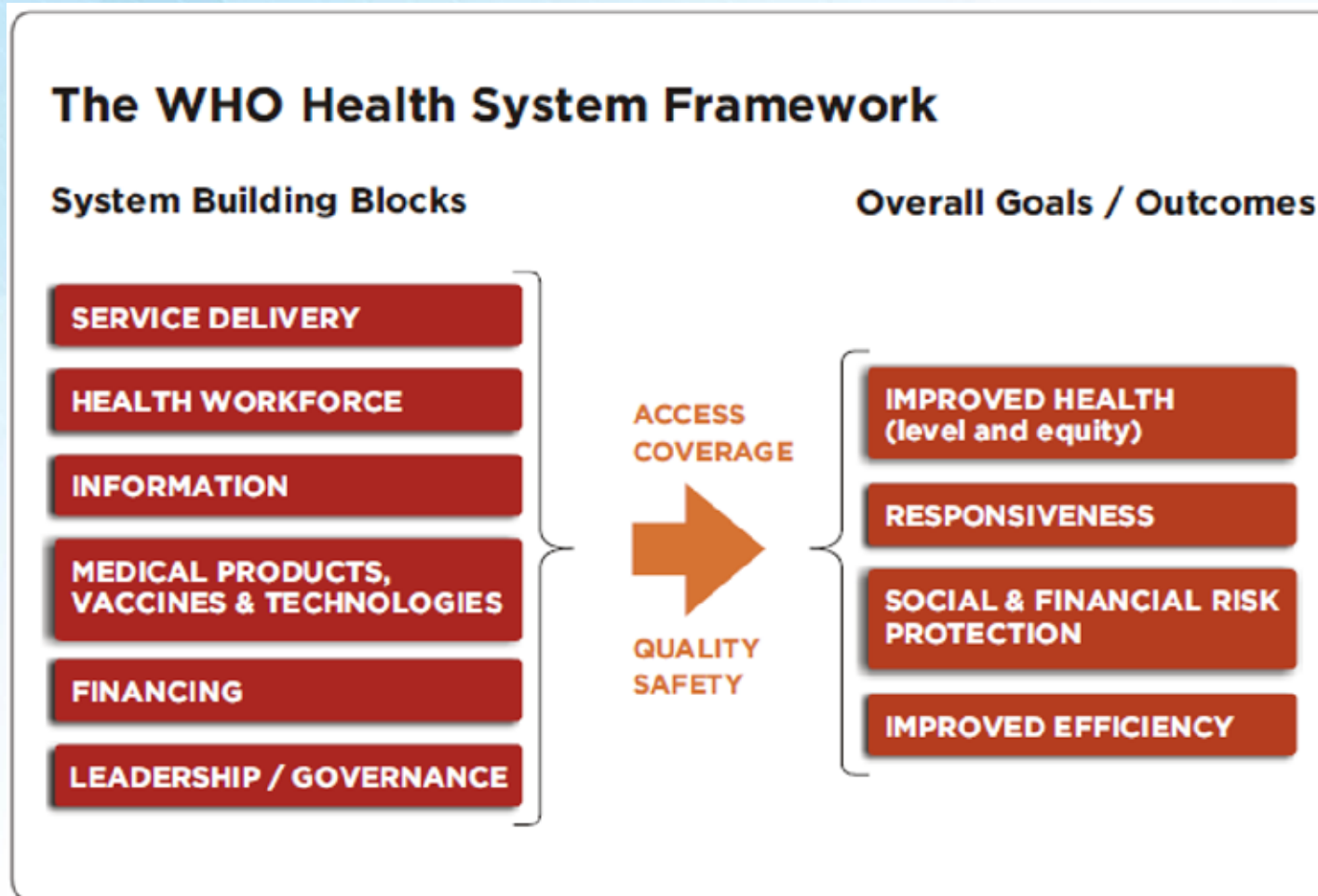
System building blocks

- Health financing: raising adequate funds for health in ways that ensure people can use needed services, and are protected from financial catastrophe or impoverishment associated with having to pay for them
- Leadership and governance: ensuring strategic policy frameworks combined with effective oversight, coalition building, accountability, regulations, incentives and attention to system design

System building blocks

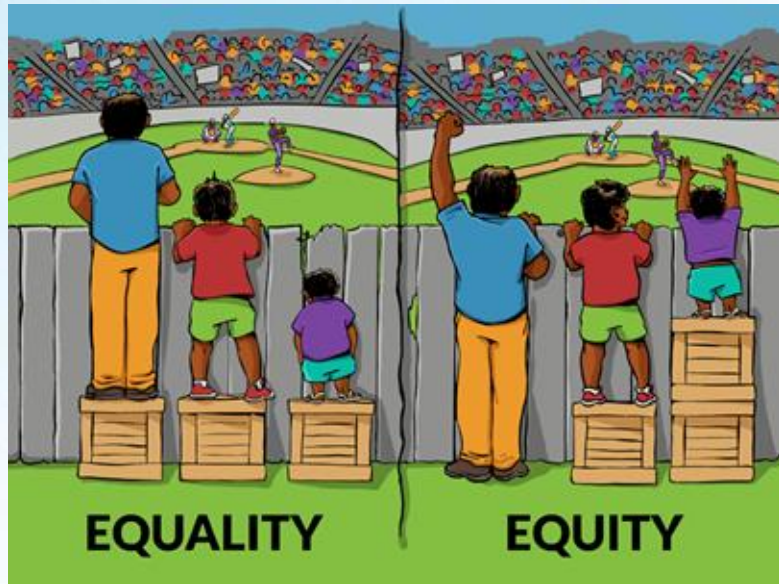
- The building blocks alone do not constitute a system, any more than a pile of bricks constitutes a functioning building
- It is the multiple relationships and interactions among the blocks – how one affects and influences the others, and is in turn affected by them – that convert these blocks into a system

Outcomes of the six building blocks



Equity

- Equity is considered part of ultimate goal of health system
- Health systems often do a very bad job at securing equity
- Equity is a fundamental reason for government intervention



EQUALITY:
Everyone gets the same—regardless if it's needed or right for them.

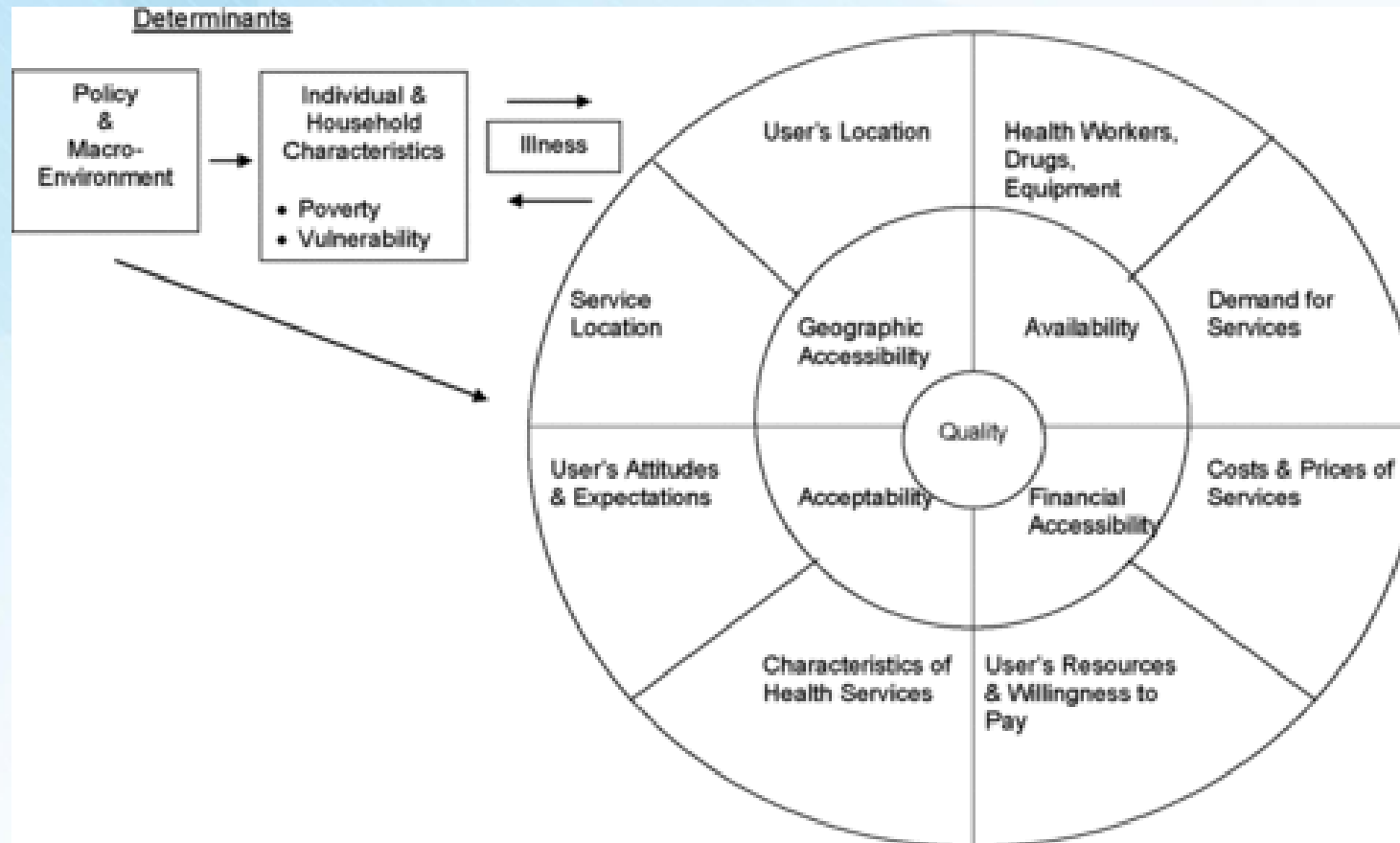


EQUITY:
Everyone gets what they need—understanding the barriers, circumstances, and conditions.



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Poverty and Access to Health Care



Peters, D. H., Garg, A., Bloom, G., Walker, D. G., Brieger, W. R., & Hafizur Rahman, M. (2008). Poverty and access to health care in developing countries. *Annals of the new York Academy of Sciences*, 1136(1), 161-171.

Access to Health Care

- Availability, the relationship of the volume and type of existing services (and resources) to the clients' volume and types of needs
- Accessibility, the relationship between the location of supply and the location of clients, taking account of client transportation resources and travel time, distance and cost

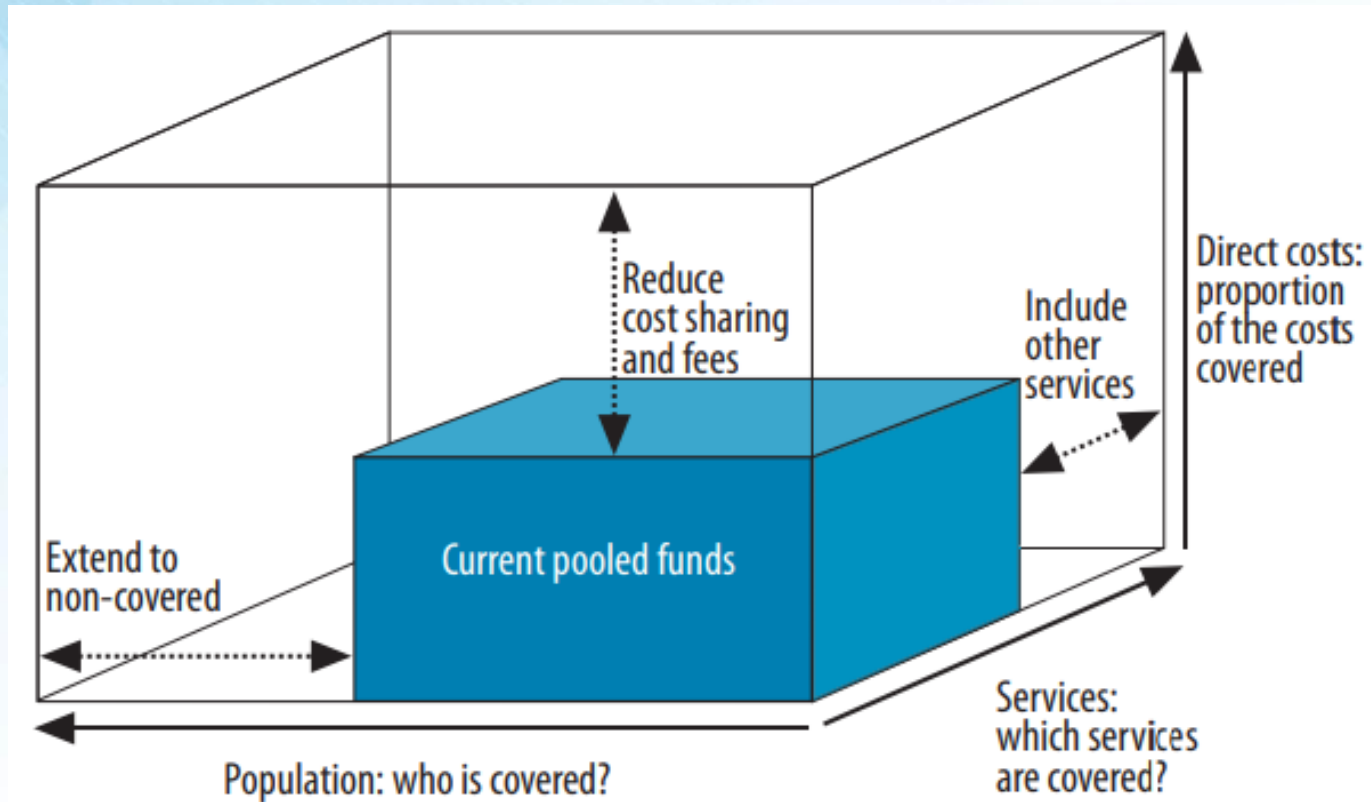
Access to Health Care

- Accommodation, the relationship between the manner in which the supply resources are organized to accept clients (including appointment systems, hours of operation, walk-in facilities, telephone service)
- Affordability, the relationship of prices of services and providers' insurance or deposit requirements to the clients' income, ability to pay, and existing health insurance

Access to Health Care

- Acceptability, the relationship of clients' attitudes about personal and practice characteristics of providers to the actual characteristics of existing providers, as well as to provider attitudes about acceptable personal characteristics

Moving Toward Universal Coverage



World Health Organization. (2010). The world health report: health systems financing: the path to universal coverage. World Health Organization. <https://apps.who.int/iris/handle/10665/44371>

แผนพัฒนาเศรษฐกิจและสังคมแห่งชาติ

- แผนพัฒนาเศรษฐกิจและสังคมแห่งชาติ ฉบับที่13 (2566-2570)
- การออกแบบแผนนี้มาจากฐานคิดรวม 4 ประการ
- 1.เศรษฐกิจพอเพียง
- 2.ความสามารถในการที่จะล้มแล้วลุกให้ไว เดินไปข้างหน้าให้ได้
- 3.เป้าหมายการพัฒนาอย่างยั่งยืนของสหประชาชาติ
- 4.การพัฒนาเศรษฐกิจชีวภาพ เศรษฐกิจหมุนเวียน เศรษฐกิจสีเขียว

แผนพัฒนาเศรษฐกิจและสังคมแห่งชาติ

- เป้าหมายหลักของแผนฯ ฉบับที่ 13 ได้กำหนดหลัก 5 ประการ
- 1.การปรับโครงสร้างการผลิตสู่เศรษฐกิจฐานนวัตกรรม
- 2.การพัฒนาคนสู่โลกยุคใหม่
- 3.การมุ่งสู่สังคมแห่งโอกาสและความเป็นธรรม
- 4.การเปลี่ยนผ่านการผลิตและการบริโภคไปสู่ความยั่งยืน
- 5.การเสริมสร้างความสามารถของไทยในการรับมือกับความเสี่ยงและการเปลี่ยนแปลงภายใต้โรคอุบัติใหม่

Health policy

- Health policy refers to decisions, plans, and actions that are undertaken to achieve specific health care goals within a society
- Policy development is more than simply having a good policy idea and shouting it from the roof tops
- Policy also requires analysis to determine:
 - Who will benefit from a policy change?
 - Who stands to lose?
 - How much will it cost?

Policy analysis workshop

- There are several ways of approaching the policy analysis
- In this workshop we will assume that the policy process is “rational” and use one particular model to approach the policy analysis: the Bardach eightfold path

Priority setting

- One of the things that policy makers need to do is to set priorities
- Policy makers cannot address each and every issue—and certainly cannot give each issue equal importance

More than a good idea

- How would you promote the implementation of this recommendation?
- What evidence is needed to show that:
 - It is a serious problem and deserves a priority
 - Solutions are possible
 - Your solution is the best possible alternative
- How would you convey the message in as succinct a way as possible?

The Bardach eightfold path



1. Define the problem

- This is the hardest and most crucial step
- What is the specific problem that needs to be addressed?
 - Be as concrete and precise as possible; one or two sentences is best
 - What needs to change and why?

1. Define the problem

- Why are you advocating for this specific policy change?
 - The problem has to be sufficiently important for the policy maker to care
 - Policy makers confront many proposals; why should he/she care about your issue?
 - What would happen if nothing changes (status quo)?

1. Define the problem

- Quantify—use too little/too much if possible
 - You want the problem to be unidirectional
 - It is difficult to quantify the middle or “sweet spot”
- Define just the problem—not the proposed remedy
- Most likely you’ll need to redefine the problem several times as you work through it

Example

- The turnover rate for Thai nurses was 41%, an all time high in the last 10 years resulting in a severe nursing shortage in a country which impact on high mortality rate, increased length of stay and decreased quality of care

Workshop 1

- Define the problem
- Writing the problem statement about
 - Overweight in elementary school student

2. Assemble some evidence

- The goal is to assemble evidence!
 - Why is this an important problem?
- Present evidence, not simply data
 - What's preferable instead: one or two important, easily remembered facts that the policy maker does not already know
- What has been tried to address the problem?
- Policy makers are not interested in old data

2. Assemble some evidence

- What will happen if nothing is done?
 - Status quo is the easiest option for policy makers to choose
 - What is wrong with the status quo?
 - What will happen if nothing is done?
 - Chicken Little—remember the policy maker has heard the “sky is falling” many times before

2. Assemble some evidence

- Collect primary or more commonly secondary data
 - Policy makers need to be persuaded with just a few facts
 - What evidence will be convincing to policy makers
 - Think of them as ADHD kids
- The value of collecting evidence is that it may:
 - Cause you to refine the problem
 - Cause you to see why prior attempts at resolving the problem have failed
 - May lead to a better set of alternatives

3. Construct the alternatives

- Provide alternative courses of action that policy makers are likely to consider
- You should not stack it so that your preferred alternative is the clear winner since policy makers will hear other ideas from other constituencies

3. Construct the alternatives

- If you ignore certain perspectives on an issue then you will be less successful
 - Include them even if you think they are wrong or if others think they are correct or valid
 - Remember Machiavelli: keep your friends close and your enemies closer

Example

- Decreasing compensation income gap
 - Persuading nurses to maintain in healthcare system
- Increasing the production of nurses
 - Decreasing workload
- Offering benefit package
 - Dealing with job stress, nurse burnout and fatigue
- Status quo

Workshop 2

- Develop 3-4 alternatives for policy makers to consider for solving overweight in elementary school student

4. Select the Criterion

- Based on prior experience, what three to four criteria do you think will policy makers use to evaluate the alternatives?
 - It is not the criteria you think they should use but what they actually will use
 - It is not public health criteria but overall criteria
 - Often the issue has been debated before—what criteria did they use previously?

4. Select the Criterion

- Benefits (i.e. Lives saved, Disabilities prevented, Dollars saved, Satisfaction, QALYs, DALYs)
- Cost
- Prevent complication
- Promote outcomes (physical, psychological, societal)
- Cost effectiveness: how much will it cost compared to the benefits of the intervention

Example

- Turnover rate
- Burnout level
- Quality of care
- Weight on each criteria by turnover rate (40%), burnout (35%) and quality of care (25%)

Workshop 3

- Identify the criterion related to selected health issue and weight percentage on each criteria

5. Project the outcomes

- What will be the costs and what will be the most likely outcomes
- What is the most likely outcome for each policy option (be realistic)
- The future cannot be told with certainty; always unintended consequences
 - Examine similar interventions
 - Develop analytic model(s)
 - Develop empirical estimates
 - Consider undesirable/unintended side effects

5. Project the outcomes

- Develop outcomes matrix
- Rank each of the possible alternatives in a matrix based on specific criterion such as cost effectiveness
- Judge each outcome on a five-point scale with the status quo getting all 0s
 - ++ means much better
 - + means better
 - 0 means same
 - - means worse
 - -- means much worse

Example

	Decreasing compensati on income gap	Increasing the production of nurse	Offering benefit package	Status quo
Turnover rate 40%	++	0	++	0
Burnout 35%	0	++	++	--
Quality of care 25%	+	-	+	0

Workshop 4

- Develop your outcomes matrix related to selected alternatives and criterion

6. Confront the trade-offs

- Is one alternative clearly dominant?
- The best choice on all criteria
- If so, you may have “stacked the deck” by selecting not very good alternatives or not considering all the relevant criteria
- Which criterion is most important?

Example

	Decreasing compensati on income gap	Increasing the production of nurse	Offering benefit package	Status quo
Effectiveness 40%	++	0	++	0
Benefit on nurse 35%	0	++	++	--
Quality of care 25%	+	-	+	0

7. Decide

- Needs to be based on the evaluation criteria and weighting of the criteria
- Why does the preferred option:
 - Have the best chance of being enacted?
 - Have the greatest impact on health care for the given number of dollars available?

Example

- I will make the decision on the alternatives that can maintain nurses in the healthcare system. I prefer offering a benefit package policy.
- After I consider other criteria including benefit on nurses and efficient **quality of care**, offering benefit package doesn't only **maintain nurses** but also advantage on **nurses well-being** and patients particularly improving the health outcome so I decide to choose offering a benefit package alternative.

8. Tell your story

- Thirty seconds; one to two sentences
 - Often called an elevator speech
- Policy maker is audience
- What is the precise message?
 - Should summarize both the problem and the proposed solution
 - Very difficult—why lobbyists are paid the big bucks!